جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ناهد فوزي عبدالعاطي عبدالمقصود
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** الهرم
رقم البطاقة : 28204042100587 **رقم التليفون :** 01112929704 **تاريخ تسجيل الإستمارة :** 15-10-2026
العمر(تاريخ الميلاد) : 04-04-1982 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمده 6 شهور [14163] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ناهد فوزي عبدالعاطي عبدالمقصود
National ID: 28204042100587

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
نوهد كس



National ID - From ID Bank
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	: الأستاذه / ناهد فوزى عبد العاطى	Request No.	: 13009211
Patient No.	: 28204042100587	Request Date	: 23/11/2024
Path No.	: 4549/24	Verification Date	: 14/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.
Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:
* Procedure: Left MRM measured 30x23x10 cm with covering skin ellipse measured 28x21 cm.
* Tumor extent: An ill defined rubbery greyish white firm tumor bed site.
* Size: 6x5x2 cm.
* Skin involvement: Nipple & areola were unremarkable grossly.
* Distance from margins: 6 cm away from deep margin, and 2 cm from skin.
* Lymph nodes number and size: Separate fibrofatty pieces measured collectively 16x11x4 cm, entangling 3 L.Ns up to 1x1 cm.
* Designation:
1-6: Tumor bed site.
7: Rest of breast cut section.
8: Deep margin.
9: Nipple.
10: Lymph nodes.
11-12: 2 more regross blocks.
* Dissected by: Dr. Samia Ali.

Microscopic Examination:
* Histological type: Residual invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Maximal tumor diameter: 0.5x0.5 cm
- Actual tumor cellularity: 5%, tumor necrosis index: 95%
* Ductal carcinoma in situ: Focal about 5%.
* Skin involvement: Paget's disease of nipple & Many positive dermal lymphatic emboli.

Verified by :
Dr.Marwa mahmoud

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ناهد فوزى عبد العاطى	Request No.	: 13009211
Patient No.	: 28204042100587	Request Date	: 23/11/2024
Path No.	: 4549/24	Verification Date	: 14/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

* Lymph node involvement: Negative for metatasis in 0/3 lymph nodes, with effect of therapy.
* Resection Margin: Free deep margin.
* Histologic response to therapy: Partial pathologic response.
* Microcalcifications: Not detected.
* Lymphovascular invasion: many positive superficial dermal lymphatic emboli.

Diagnosis:
Left breast cancer post NACT, MRM specimen:
Residual invasive duct carcinoma, grade II, with focal insitu component 5% and many positive superficial lymphatic emboli.
- Free deep margin.
- Paget's disease of nipple.
- Free lymph nodes (0/3) with therapy effect.
- Partial pathologic response to therapy.
- Stage: ypT4, ypN0.
- RCB: 1.647, RCB class: RCB-II.

Signature
Dr. Marwa Mahmoud
Dr. Yasmine Essam

Verified by :
Dr.Marwa mahmoud

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Code:	0117-8320	Age:	41 y, 3 d	Date:	20/04/2024
Name:	Nahed Fawzy Abd El Aty		Refe... , Prof. Dr.:	no physician	

POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN & PELVIS WITH CORONAL RECONSTRUCTION: -

- **Status known case with left breast cancer for staging.**

Findings:

I- CHEST

- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No CT evidence of enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Normal cardiac size and shape. No pericardial effusion detected.

II- ABDOMEN & PELVIS

- **The liver enlarged in size with homogenous CT texture. No focal lesions.**
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renals.
- No abdominal lymph node enlargement seen.
- No ascites or abdominal collections noted.
- Unremarkable pelvic organs

OPINION:

- **Hepatomegaly.**
- **Otherwise unremarkable CT study with no sizable metastatic deposit.**

Kindly Accept My Best Wishes,
AMR IBRAHIM, MD

CT Scan
[[DOCTYPE.CT_SCAN]]

Prevention Program for Women's Health
Ministry of Health and Population

البرنامج الوطني لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : نانسي فوزي عبدالعاطي عبدالمقصود Sex: Female
2-ID number: 28204042100587
3- Medical number: 83199845
Old Med :
Patient Age : 42 Y
11/6/2024

Family history
Phone 01012929704
BMI
REASON OF MDT
marital status
NRS Score 2
GROUP
Date of MDT 01/07/2024
Co morbidity
Menopausal status
Initial STAGE

Clinical data

Breast Imaging :
CT (BIRADS4) path L.N

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	6/8	8/8	0		50%

T	N	N
4d	3a	0 by ct

Sonomammography

PATIENT HISTORY
1- Bone scan.
2- Echo
3- Full lab

MDT OUTCOME & PLAN

meanwhile start NACT (Dose dense AC *4 with GSF support then taxotere *4)
guided by clinical response then consider LT MRM (if proven non mets)

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

جوستاف روسي

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

09/16/2026

MDT FORM

Name:	Nahed Fawzy Abd Elaaty	**MRN:**	1468
Age:	44 Y	**ID:**	
Menopausal Sta	pre	**Phone:**	01112929704
NRS SCORE:	1	**Marital Status:**	married
BMI:		**Family History:**	mother:breast
CO Morbidity:	free	**Reason Of MDT:**	

Clinical Data:

BX: IDC

ER: 6/8 **PR:** 8/8 **HER2:** 0 **KI67:**

Sonomammography:

Mets Workup:

History:

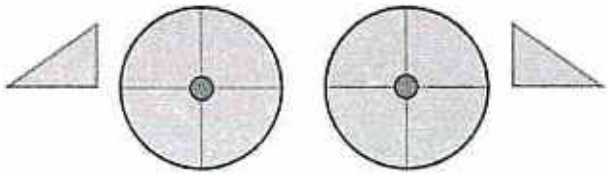
44 y/o female, ER 6/8, PR 8/8, HER2 ?ve. Initially cT4d N3a (locally advanced IBC). Received NAC: 2 cycles AC ? 4 cycles Docetaxel ? THEN 2 cycles AC.
11/2024 : Underwent Lt simple mastectomy + SLNB. Final pathology: ypT4 N0, RCB-II (partial pathological response), with residual disease. then PORT, then adjuvant AI +zoladex+zometa
Now presents with incisional lesion, suspicious for LRR
- CT CAP on 05-05-2026:
1- few rt prominent axillary LNs , largest 20*17mm
2- RT femoral & left iscial small sclerotic lesions likely bone island

SM ON 05-05-2026:
left two echoic lesions , mesuring 15*4 & 26*5.5 mm, for biopsy
right (BIRADS III), UOQ microcalcification for FUP

---7/2026 bx :
invasive mammary adenocarcinoma
er 8/8
pr 0/8
her2 0
ki 35%

8/2026 bone scan :worrisom lower sternal lesion

R L



TNM

Recommendations:

MDT Outcome & Plan

Bring results of CT CAP & bone scan to review...and if feasible, go for PET/CT scan

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د شيماء لاشين	د امنيه عيسوي				

MDT Form
[[DOCTYPE:MDT_FORM]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Nahed Fawzi Abdel-Aty
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 31-08-2026 11:48
Medical No. : 451159

NUCLEAR MEDICINE BONE SCAN REPORT

44-year-old female with left breast carcinoma, status post left total mastectomy (November 2024). Referred for follow-up skeletal evaluation.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- A focal area of mild-to-moderate increased radiotracer uptake is noted at the lower sternum/xiphisternal region; indeterminate—benign (prior trauma, sternochondritis, or sternocostal degenerative changes) versus early solitary metastasis.
- Subtle faint activity is noted in the distal left femur, likely non-specific periarticular/degenerative.
- The remainder of the axial and appendicular skeleton shows normal physiological tracer distribution without focal lesions.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Worrisome lower sternal uptake, for radiological correlation to exclude the possibility of metastatic deposit therein.
- No other osseous lesions depicted.

١٤٦٨

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name	ناهد فوزي عبدالعاطى	**Case No.**	**275745**
Sex/Age	**Female 44 Y**	**Patient No.**	**20587**
Registered Date	**10/6/2026 11:02:12 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**05/7/2026 05:06:40 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**21/7/2026 10:32:22 AM**	**Pathology No.**	**1823/26**

Clinical Data: Left breast operative bed lesion in known case of left breast cancer and left MRM.

Nature of Specimen: Tru-cut biopsy, collected on 10/06/2026, at 11:10 am. Cold ischemia time is 20 seconds.

Technique:
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic examination:
- Histopathological examination reveals cores of breast tissue showing positive dermal involvement by invasive mammary carcinoma.

Results:
*** IPT (All controls showed appropriate reactivity):**
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).
- Ki67: 35%

Conclusion:
Left breast operative bed lesion, Tru-cut biopsy:
- Positive for dermal involvement by invasive mammary carcinoma.

IPT:
- ER: Positive (score 8/8).
- PR: Negative (score 0/8).
- Her2: Ultra-Low (score 0+).
- Ki67: 35%

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
** ***Scoring system: "Her2"***

Patient Name	ناهد فوزي عبدالعاطى	**Case No.**	**275745**
Sex/Age	**Female 44 Y**	**Patient No.**	**20587**
Registered Date	**10/6/2026 11:02:12 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**05/7/2026 05:06:40 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**21/7/2026 10:32:22 AM**	**Pathology No.**	**1823/26**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim.

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

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Patient Name Nahed Fawzy	Date 05 / 05 / 2026
Exam : Ultrasound ABDOMEN & PELVIS	

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

The liver
Is enlarged in size measures 17.5cm at Right MCL and showing bright homogenous echo pattern.
No definite focal lesion is seen. No intrahepatic biliary dilatation is noted.
The Portal vein is patent.

The gallbladder No Stones or mud
The common bile duct is normal in caliber.

The pancreas No gross mass.

The spleen is average in size, homogenous echo pattern, No focal lesions.

Both kidneys are normal in size and shape and echogenicity, normal parenchymal thickness and CM difference.
No stones or back pressure changes on both sides.
No dilated pelvicalyceal system,

The urinary bladder is suboptimal filled, no mass or stones.

Uterus and adnexa
No adnexal mass or free fluid is seen.
No ascites is noted.

OPNION:
Bright hepatomegaly most likely fatty liver for lab correlation

Best regards
Dr. Mahmoud Ashour

Ultrasound
[[DOCTYPE ULTRASOUND]]

Patient's Name:	**NAHED FAWZY ABD EL AATY**
Date:	**2026-05-05**
Referred by:	Prof.Dr. **CT CAP+C**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported left breast carcinoma , compared to last study dated 24/1/2025, the current study revealed .

(I) Chest:

- Left mastectomy with clear operative bed .
- Few prominent right axillary lymph nodes , the largest measuring 20x 17 mm
- No sizable pulmonary no
- No sizable hilar or mediastinal lymph nodes .
- Clear both lung fields .no sizable pulmonary masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density ,no enhancing focal lesion
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Right femoral & left ischial small sclerotic lesions likely bone island .
- No worrisome lytic or sclerotic bony lesions
- No vertebral structural collapse .

OPINION:.

- Overall stationary course

Dr.Reham Khedr, MD

CT Scan
[[DOCTYPE.CT_SCAN]]

Patient's Name:	NAHED FAWZY A. EL ATY
Date:	2026-05-05
Referred by:	Prof.Dr.
Hospital:	**Goustave Roussey International Egypt**

LEFT DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION SONOGRAPHY REVEALED:

CLINICAL SUMMARY:
Left MRM with two small operative bed ulcerations

FINDINGS:

LEFT MASTECTOMY BED.
Two hypoechoic lesions are seen underlying the previous scar and clinically noted ulcerations, laying within the dermal layers, measuring 15x4 and 26x5.5mm.
No definite pathological regional lymph nodes.

RIGHT BREAST:
Fairly dense fibro-glandular densities, (ACR b).
No spiculations masses or distortion.
Faint rounded micro calcifications are seen in the UOQ.
Complimentary **U/S** demonstrated no solid or cystic lesion.
Normal skin/nipple/areola complex.
Non-specific axillary nodes.

CONCLUSION:
- Two left operative bed hypoechoic dermal lesions, for biopsy to exclude recurrence.
- Right breast (BIRADS 3): UOQ micro calcifications, for close follow up.

Best regards,
Dr. Mahi Mokhtar

Mammogram
[[DOCTYPE MAMMOGRAM]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Name: Mrs. Nahed Fawzey Abdulatea Abdulmaksoud
Date: 2025-12-05

Right Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Post left MRM & ALAND, routine follow up.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 1): normal study.
Clear left mastectomy bed and axilla.

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM / /20 G A

Name	ناهد نوراى + العاقى	MRN	1164 Mam
		ID	٨٢١٩٩١٤٥
Age	42	Phone	01112929704
Menopausal status	Post.	Marital status	married
NRS SCORE		Family history	+ve mother
BMI		REASON OF MDT	١١١٤٩٢٤٤٥
CO morbidity	free		

Clinical Data

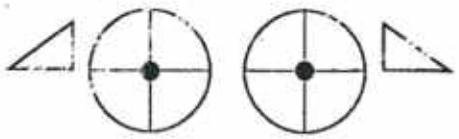
BX: IDC lt MRM

ER:	PR:	HER 2:	Ki67:
6/8	8/8	0	5%

Sonomammography: T4d N3 A Mo

Mets Workup:

R L



TNM

Recommendations:

Post Lt simple mastectomy + SLNB

IDC GII → free margins. + Paget's of nipple disease

LN → 0/5

(AC x4 Tx) Post NACth → partial pathological Response RCB II

yT4 No

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM / / 20

G ١١٦٤

Name	[illegible] فوزى عبد العاطى عبد المقصود	MRN	٨٣١٩٩٨٤٥
		ID	
Age	42y	Phone	٠١٢٢٩٢٩٧٠٤
Menopausal status	Perimenop	Marital status	
NRS SCORE		Family history	٠١١٠٩٧٠٩٢٥
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

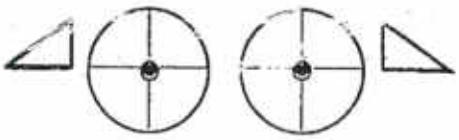
BX: IDC

ER:	PR:	HER 2:	Ki67:
6/8	8/8	3+	50%

Sonomammography:

Mets Workup:

R [illegible] PR L



TNM

T4

Recommendations:

(L) Locally adv. Brst C.
Rem'd NACT 2AC 4TxT then 2AC
PR → for MRM

MDT OUTCOME & PLAN

At MRT as pe mutual dec.

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذه / ناهد فوزى عبد العاطى	Request No.	: 13005664
Patient No.	: 28204042100587	Request Date	: 20/03/2024
Path No.	: 1116/24	Verification Date	: 06/04/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 20/3/2024, 9:40 am.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 60 % of tumor cells (score 6/8).
- PR: Strong nuclear reaction in 70 % of tumor cells (score 8/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67: 50 % nuclear reaction of tumor cells.

Conclusion:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with extensive ductal carcinoma in situ (DCIS).
IPT:
- ER: Positive (score 6/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 50% of tumor cells.

* Scoring system: "ER, PR"

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذه / ناهد فوزى عبد العاطى	Request No.	: 13005664
Patient No.	: 28204042100587	Request Date	: 20/03/2024
Path No.	: 1116/24	Verification Date	: 27/03/2024
Sex	: Female	Financial Category	: اور ام دار السلام - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 20/03/2024, 09:40 am.

Gross Examination:
- Multiple grayish white tissue cores, up to 1.5 cm long, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: Extensively detected.
* Perineural invasion: Not detected.
* Angioinvasion: Not detected.

Diagnosis:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, With extensive ductal carcinoma in situ (DCIS).
- For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Iman Gouda.
Dr. Muhamed Turkey.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Code:	0117-9320	Age:	41 y, 3 d	Date:	20/04/2024
Name:	Nahed Fawzy Abd El Aty		Referred by Prof. Dr.:	no physician	

POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN & PELVIS WITH CORONAL RECONSTRUCTION: -

- **Status known case with left breast cancer for staging.**

Findings:

I- CHEST

- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No CT evidence of enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Normal cardiac size and shape. No pericardial effusion detected.

II- ABDOMEN & PELVIS

- **The liver enlarged in size with homogenous CT texture. No focal lesions.**
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renals.
- No abdominal lymph node enlargement seen.
- No ascites or abdominal collections noted.
- Unremarkable pelvic organs

OPINION:

- **Hepatomegaly.**
- **Otherwise unremarkable CT study with no sizable metastatic deposit.**

Kindly Accept My Best Wishes,

Dr. **AMR IBRAHIM, MD**





National ID - Front & Back
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-١١٢٩٢٩٧٠٤

١١٢٤

مع حلفه

مبادرة


٢٨٢٠٤٠٤٢١٠٠٥٨٧
٢٠٢١/١٢
ربة منزل
انثى مسلمة متزوجة
محمد ابراهيم على عبدالسلام
البطاقة سارية حتى ٢٠٢٨/١٢/٢٦
Nasional ID – Back
[DOCTYPE:ID_BACK]

JSTAVE / OUSSY
RNATIONAL

جوستاف روسي انترناشونال

ECHO REPORT

NAME : DATE: 13/1/2026

ID NUMBER : SEX : AGE:

F	60%	55-75%	PWT		0.6-1.1cm
VEDD	4.8	3.5-5.8cm	AO.R		3-3.7cm
VESD	2.7	2-4cm	LA		2-4cm
S		25-45%	RV		0.7-2.3com(supine)
WT		0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions
LV Wall Thickness
LV Diastolic Function DDJ EF= 60%
SWMA no
LA and Ao Root Dimensions
Mitral Valve
Aortic Valve
Tricuspid Valve
Pulmonary Valve
Right Side and Systolic Function
Intracardiac Masses and Effusion no pericardial effusion

lt pleural effusion

DR

جوستاف روسي

7/10

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

10/07/2026

MDT FORM

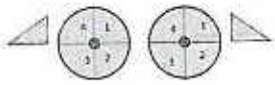
Name:	Nahed Fawzy Abd Elaaty	**MRN:**	1468
Age:	45 Y	**ID:**	28204042100587
Menopausal Sta	pre	**Phone:***	01112929704
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	recurrence suspected mets
Surgical History:*		**ECOG Performance Status**	○ 0 ◉ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX:

ER: 6/8 **PR:** 8/8 **HER2:** - **KI67:**

R L



T N M

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

History:

44 y/o female, ER 6/8, PR 8/8, HER2 -ve. Initially cT4d N3a (locally advanced IBC). Received NAC: 4 cycles AC 4 cycles Docetaxel 11/2024 : Underwent Lt simple mastectomy + SLNB. Final pathology: ypT4 N0, RCB-II (partial pathological response), with residual disease. then PORT, then adjuvant AI + zoladex + zometa

Now presents with incisional lesion, suspicious for LRR

· CT CAP on 05-05-2026:

1- few rt prominent axillary LNs , largest 20*17mm

2- RT femoral & left iscial small sclerotic lesions likely bone island

---7/2026 bx from chest wall nodule: invasive mammary adenocarcinoma er 8/8 pr 0/8 her2 0 ki 35%

8/2026 bone scan :worrisom lower sternal lesion indeterminate, for correlation

Clinical Staging:

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:

NEW CT CAP requested, bone scan films submitted

Recommendations:

last MDT > Bring results of CT CAP & bone scan to review

for remdt with review of radiology as recommended by dr Nada

MDT Outcome & Plan

after reviewing new CT CAP 1/10/2026, a case of recurrent right breast IDC with pulmonary, nodal and osseous deposits, shift to CDK4/6i + faslodex + zoladex + zometa q3 m

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د شيماء لاشين	د امنيه عيسوي		

7/10/2026 7/10/2026